

Blood and Blood Product Administration

Transfusion Safety, Consent, and Adverse Reaction
Management

Regulation: 42 CFR §482.27 | §482.25

POLICY OVERVIEW

Blood and blood product administration carries inherent risks including transfusion reactions, infectious disease transmission, and clerical errors leading to ABO incompatibility. Robust policies ensure safe transfusion practice from specimen collection through post-transfusion monitoring.

KEY REQUIREMENTS

1 Pre-Transfusion Requirements

Informed consent for blood transfusion must be obtained and documented before non-emergent transfusions. A current type and screen (within 3 days for non-pregnant patients, per facility policy) must be on file. Two-person verification of the blood product and patient identification is mandatory.

2 Patient Identification at Bedside

Before hanging any blood product, two licensed staff members must independently verify: patient name, date of birth, MRN, blood type, unit number, expiration date, and any special processing requirements at the bedside using the patient's armband.

3 Transfusion Administration

Blood products must be administered through an appropriate filter within 4 hours of issue from the blood bank. Vital signs must be taken before the transfusion, 15 minutes after initiation, and upon completion. The first 15 minutes must be observed by a nurse.

4 Transfusion Reaction Management

If a transfusion reaction is suspected (fever, chills, urticaria, back/flank pain, hemoglobinuria, hypotension), the transfusion must be stopped immediately, IV line kept open with NS, physician notified, and the blood bank notified to begin reaction workup.

5 Blood Bank Inventory Management

Policies must address emergency release procedures, massive transfusion protocols (MTP), autologous blood handling, and management of patients who refuse blood products for religious reasons.

COMPLIANCE NOTES

The transfusion committee must review all transfusion reactions, near-misses, and utilization patterns. Blood utilization review should assess appropriateness of transfusion triggers. The blood bank medical director must be a licensed physician with appropriate qualifications.

COMMON SURVEY CITATIONS

Cited under §482.27(a) – Blood bank or transfusion service standards, and §482.27(b) – Blood product availability and safety.